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Health Surveillance Brief: Obesity Among Adults in Ontario.

Introduction:

Obesity is a major public health concern in Ontario, which affects a significant part of the adult population. This report aims to help physicians, nurses, and doctors with a clear understanding of obesity trends, risk factors, and their health impacts of adults (18+) in Ontario. By examining the disease using descriptive methods of person, place, and time, we can better understand who is most affected and where the issue is most prevalent, and how the trends have changed over time. The goal of this report is to promote their response to prevention and treatment strategies.

Obesity is defined as an excessive accumulation of body fat that increases the risk of various health complications. It is usually measured using the Body Mass Index (BMI), where a BMI of 30 or higher indicates obesity. The condition is influenced by multiple factors, including genetics, lifestyle choices, socioeconomic status, and environmental factors. Understanding obesity in Ontario's adult population is very important for developing effective healthcare strategies and treatment.

Descriptive Study:

Person (Who is Affected?)

Obesity affects a large portion of Ontario's adult population, with certain groups being more vulnerable. Adults aged 45–64 have the highest prevalence (36%), followed by those aged 25–44 (28%) (Thielman et al., 2018). Men are slightly more likely to be obese than women (32% to 28%), but women face more challenges, such as weight stigma in healthcare or the environment, which delays treatment (Public Health Ontario, 2020). Socioeconomic factors also play a huge role as low-income communities and racialized groups (e.g., Indigenous, Black, and South Asian populations) experience higher obesity rates due to limited access to affordable nutritious food, higher stress level, no healthcare access and spaces for exercise (Alter et al., 2012). 40% of adults in low-income neighborhoods report obesity compared to 25% in higher-income areas (Alter et al., 2012). Additionally, individuals with existing health conditions such as diabetes or heart disease are more likely to experience obesity-related complications (Alter et al., 2012).

Place

Obesity rates in Ontario vary by geographical location, with their urban and rural differences playing a huge role. Walkability is a crucial factor that influences obesity rates, and they found Individuals residing in neighborhoods with higher Walk Scores tend to have lower obesity rates due to increased engagement in physical activities, which includes walking for daily activities such as commuting and errands. areas with lower walkability also had higher obesity rates due to relying on vehicles and reduced physical activity. (Chiu, 2015) Urban cities like Toronto and Ottawa have better infrastructure, including sidewalks, parks, and public transit, which encourage physical activity. However, even in these cities, neighborhoods with lots of fast-food outlets and limited access to affordable, healthy food options still experience increased obesity rates. In conclusion, areas lacking these infrastructure tend to see increased obesity levels. (Kaczynski et al. 2019)

Public Health Ontario (2024) also emphasized weight stigma in healthcare access, as individuals in low-walkability areas may face additional barriers when seeking medical support for weight-related problems. Lastly, (Statistics Canada, 2015) supports the findings that individuals in high-walkability environments are less likely to be obese, these shows the importance of urban planning and community design in reducing obesity rates. These studies show that improving walkability and access to active transportation could play a huge role in reducing obesity in Ontario.

Time

Obesity rates in Ontario have increased over the years. More adults are now obese compared to past decades, Obesity rates rose from 24% (2010) to 31% (2019) (McDiarmid, 2019) and by 2035 the rate could reach 38%, increasing healthcare costs by $1.2 billion/year (Katzmarzyk, 2011). This is because people are eating more processed foods and moving less. Many jobs today require long hours of sitting, and screen time has replaced outdoor activities. These lifestyle changes make it easier for people to gain weight and harder for them to stay active. Obesity is not just about weight. It also increases the risk of diabetes, heart disease, and mental health problems like depression (Alter et al., 2012). Treating these health issues costs Ontario $2.1 billion every year (Katzmarzyk, 2011). and many people with obesity face judgment from others even doctors, which makes them avoid seeking help (Public Health Ontario, 2024). Reduced physical activity during lockdowns likely worsened obesity (Campitelli et al., 2013).

Healthcare workers should measure BMI at every visit to catch health risks early and use kind, non-stigmatizing language when discussing weight. They should also advocate for policies like creating free or low-cost gym access in low-income neighborhoods and improving rural walkability by adding sidewalks, parks, and safe walking paths. They should also encourage simple actions like encouraging daily walks or cooking at home.

Health Implications

Obesity is associated with serious health risks, including cardiovascular disease, type 2 diabetes, and mental health challenges (Alter et al., 2012). It also contributes to increased healthcare costs and hospital admissions (Katzmarzyk, 2011). They should prevent this by focusing on promoting healthy eating, encouraging physical activity, and addressing social factors that contribute to obesity. Healthcare also providers play a critical role in early detection, patient education, and advocating for good policies. Additionally, obesity also impacts mental well-being which leads to higher rates of depression, anxiety, and isolation.

Many people experience weight-related discrimination and stigma, which can prevent them from seeking medical care and performing healthier behaviors. Physicians and healthcare workers must be compassionate and supportive to patients struggling with obesity. Economically, obesity costs Ontario billions annually through direct healthcare spending. Obesity also promotes health inequities. 40% of obese Ontarians report experiencing stigma in healthcare settings, which discourages preventive care and worsens mental health outcomes (Public Health Ontario, 2024). Lower-income and less-educated populations face higher risks due to limited access to affordable healthy food, safe outdoor spaces, and supportive and preventive programs (McDiarmid, 2019).

Conclusions

Obesity remains a public health issue among adults in Ontario, with high implications for individuals and healthcare systems. Understanding the descriptive study through person, place, and time helps in identifying vulnerable populations and acting accordingly. Addressing obesity requires a combination of clinical care, policy efforts, and government strategies to reduce its prevalence and improve overall health of the population. Healthcare professionals also play a vital role in the fight against obesity by guiding patients toward healthier lifestyles and advocating for policy changes that promote wellness. By addressing the root causes of obesity and implementing comprehensive

prevention programs, we can work towards reducing its long-term impact on Ontario's healthcare system and overall population health.

CONCLUSIONS

[Thielman, J., Harrington, D., Rosella, L. C., & Manson, H. (2018). Prevalence of age-specific and sex-specific overweight and obesity in Ontario and Quebec, Canada: a cross-sectional study using direct measures of height and weight. BMJ Open, 8(9), e022029–e022029. https://doi.org/10.1136/bmjopen-2018-022029]

https://www.publichealthontario.ca/-/media/Documents/O/24/obesity-weight-stigma-health-promotion.pdf?rev=5fe941d354494c38b7871bfd4146146e&la=fr

McDiarmid, C. (2019, June 25). This is a Health fact sheet about Canadians aged 18 and older who are overweight or obese, based on self-reported height and weight that has been adjusted with correction factors. The results shown are based on data from the Canadian Community Health Survey. Statcan.gc.ca; Government of Canada, Statistics Canada. https://www150.statcan.gc.ca/n1/pub/82-625-x/2019001/article/00005-eng.htm]

Alter, D. A., Wijeysundera, H. C., Franklin, B., Austin, P. C., Chong, A., Oh, P. I., Tu, J. V., & Stukel, T. A. (2012). Obesity, lifestyle risk-factors, and health service outcomes among healthy middle-aged adults in Canada. BMC Health Services Research, 12(1). https://doi.org/10.1186/1472-6963-12-238]

Katzmarzyk, P. T. (2011). The Economic Costs Associated with Physical Inactivity and Obesity in Ontario. Health & Fitness Journal of Canada, 4(4), 31–40. https://doi.org/10.14288/hfjc.v4i4.112]

Campitelli, M. A., Rosella, L. C., & Kwong, J. C. (2013). The association between obesity and outpatient visits for acute respiratory infections in Ontario, Canada. International Journal of Obesity, 38(1), 113–119. https://doi.org/10.1038/ijo.2013.57

Chiu. (2015). Walk Score® and the prevalence of utilitarian walking and obesity among Ontario adults: A cross-sectional study. Health Reports, 26(7). https://pubmed.ncbi.nlm.nih.gov/26177041/